Bipolar is a difference in how the brain regulates mood and energy — not moodiness, and not a personality flaw. Whether this belongs under the neurodivergent umbrella is genuinely debated. Many people who have it identify as neurodivergent; some prefer not to. Both are reasonable, and nobody gets to decide it for you.

In plain words

Bipolar involves episodes of elevated mood and energy (mania or the milder hypomania) alternating with periods of depression. Episodes last weeks or months, not hours, and they aren't triggered by ordinary good or bad days. It's lifelong, and is said to be manageable with the right treatment and support.

What it can feel like from the inside

- Energy and confidence arriving in a wave that feels wonderful and is genuinely dangerous.
- Sleeping three hours and feeling brilliant — the earliest reliable warning sign.
- Making plans and purchases during a high that make no sense afterwards.
- Then a flatness that isn't sadness so much as everything being switched off.
- Some people have described grieving the version of themselves that existed during the high.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"Everyone has mood swings."	Mood swings last hours. Episodes last weeks, and reorganise your whole life.
"Mania is just being happy."	It impairs judgement, sleep and safety. Plenty of it is frightening rather than fun.
"They're fine now, so it's over."	Stable is the goal and often achieved. It doesn't mean it's gone.
"They should come off the medication."	Stopping is the most common route back to crisis. Never advise this.
"It's so bipolar of the weather."	Using it as a synonym for changeable makes the real thing harder to disclose.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Genuine creativity and originality of thought.

Deep empathy for people at the bottom of things.

Considerable resilience — this takes real strength to manage.

Insight into their own mind that most people never develop.

Energy and drive that, well managed, achieves a great deal.

What actually helps

- ☐ Learn their early warning signs with them
- ☐ Support the treatment, don't second-guess it
- ☐ Stay in touch during the low ones
- ☐ Take sleep changes seriously
- ☐ Don't make big decisions during a high
- ☐ Treat stability as an achievement

If someone tells you this is them

HELPFUL

"I've noticed you're sleeping less — how are you doing?"

"I'm around either way."

"What helps when it's like this?"

LESS HELPFUL

"You seem manic!" (as a joke)

"Do you still need those tablets?"

"Snap out of it."

Not moodiness. A different system for regulating energy, which can be managed well.

If you are struggling right now, you don't have to wait until it gets worse. In the UK you can call Samaritans free on 116 123, any time, or contact your GP. Elsewhere, your local crisis line or doctor is the place to start.

A friendly explainer written for the community — not a diagnostic tool and not medical advice. If you want an assessment, your GP is the place to start. Nobody experiences this identically, so take what fits and leave the rest.

Free to share — print it, forward it, hand it to anyone who needs it. More free guides, one per neurotype, at froglogic.co.uk